SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Encorafenib and binimetinib for advanced BRAF-mutated melanoma

NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation. · NCCP 00563 · version 4 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

Schedule
Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests, ECG and heart-function monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Encorafenib
ORAL BRAF INHIBITOR

- ☐ Skin reactions and new skin tumours
- ☐ Nausea, vomiting, fatigue and joint pain
- ☐ Bleeding and eye inflammation
- ☐ Heart-rhythm and liver changes
- ☐ Harm to pregnancy and interactions

Binimetinib
ORAL MEK INHIBITOR

- ☐ Reduced heart pumping function
- ☐ Retinal/eye changes
- ☐ Raised CK and muscle injury
- ☐ Liver changes, diarrhoea and rash
- ☐ Lung inflammation, clots and bleeding
- ☐ Harm to pregnancy

Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00563 v4.

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